A Plus Home Health Systems

OASIS

5670 W Flamingo Rd ste d
Las Vegas NV 89103
Phone: (702)720-7929 Fax: (702)342-8398
NPI: 1154824985

STAFF INFO
Name: Rudich, Charlotte RN
Patient INFO
Name: Tuia, Alfred S.
DOB: 1978-12-02

OASIS Assessment: OASIS E2 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number: 297256

(M0014) Branch State: Branch State: **Nevada**

(M0016) Branch ID Number:

(M0018) Attending Physician (NPI): 1083392765

Physician: NUNEZ CENDAN, CARLOS

Physician Address: 6930 S CIMARRON RD STE 260 LAS VEGAS NV 89113

☐ Not Applicable

(M0020) Patient ID number: 178779

(M0030) Start of Care Date: 04/08/2026

(M0032) Resumption of Care Date: mm/dd/yyyy

☑ Not Applicable

(M0040) Patient Name:

First Name:	Middle Name:	Last Name:	Suffix:
Alfred	S	Tuia	

(M0066) Birth Date: 12/02/1978

(A0810) Sex: ● 1. Male ○ 2. Female

(M0050) Patient State of Residence: State of Residence: **Nevada**

(M0060) Patient Zip Code: 89102

(M0063) Medicare Number: 7G11WM3DR13

☐ Not Applicable

(M0064) Social Security Number: 500825230

☐ Not Applicable

(M0065) Medicaid Number: 00001905731

☐ Not Applicable

(M0080) Discipline Completing Assessment: ● 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Rudich, Charlotte RN

(M0090) Date Assessment Completed: 04/08/2026

(M0100) Assessment Reason: ● 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply:

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☐ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☑ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language:

A. What is your preferred language? English

B. Do you need or want an interpreter to communicate with a doctor or health care staff? ● 0. No ○ 1.Yes ○ 9. Unable to determine

(B1300) Health Literacy:

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

○ 0. Never ● 1. Rarely ○ 2. Sometimes ○ 3. Often ○ 4. Always ○ 7. Patient declines to respond ○ 8. Patient unable to respond

(M0102) Date of Physician-ordered Start of Care (Resumption of Care):

If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.

Skip to A1255, Transportation, if date entered:

04/08/2026 ☐ NA - No specific SOC/ROC date ordered by physician

(M0104) Date of Referral:

Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:

mm/dd/yyyy

(M0150) Current Payment Sources for Home Care:

(Mark all that apply)

- ☐ 0. None; no charge for current services
- ☑ 1. Medicare (Traditional fee-for-service)
- ☐ 2. Medicare (HMO/managed care/Advantage plan)
- ☐ 3. Medicaid (Traditional fee-for-service)
- ☐ 6. Title programs (for example, Title III, V, or XX)
- ☐ 7. Other government (for example, TriCare, VA)
- ☐ 8. Private insurance
- ☐ 9. Private HMO/managed care

- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1255) Transportation (NACHC©):

In the past 12 months, has lack of reliable transportation kept you from medical appointments, meetings, work or from getting things needed for daily living?

- [x] 0. Yes
- [ ] 1. No
- [ ] 7. Patient declines to respond
- [ ] 8. Patient unable to respond

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(M1000) Inpatient Facilities that patient was discharged within the past 14 days?

(Mark all that apply)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [x] NA - Patient was not discharged from an inpatient facility

(M1005) Inpatient Discharge Date (Most Recent):

mm/dd/yyyy [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity (POC Element):

The patient, Alfred Tuia, a 47-year-old male with a primary diagnosis of Spondylosis w/o myelopathy or radiculopathy, lumbar region and a secondary diagnosis of bilateral primary osteoarthritis of the knees, presents significant medical and functional challenges that necessitate admission to home health care. His medical history is notable for chronic bilateral knee pain, for which he has undergone multiple pain management interventions, including radiofrequency ablation and knee injections. The patient experiences persistent pain levels averaging 7/10, with the worst pain reaching 9/10, severely impacting his daily activities and quality of life. He has reported limited range of motion in his left shoulder, indicative of adhesive capsulitis, alongside ongoing difficulties with mobility due to his knee conditions.

Given the complexity of his condition, the patient requires skilled nursing care for pain management, medication management, and monitoring of his response to ongoing therapies. Additionally, physical therapy is essential to improve his functional mobility, enhance his strength, and address the limitations imposed by his orthopedic issues. The necessity for home health services is further underscored by the patient's recent history of pain management procedures and his current requirement for support in managing chronic pain effectively.

Furthermore, the patient's status post multiple interventions, including bilateral knee radiofrequency ablation and ongoing treatment for lumbar spondylosis, indicates a need for close monitoring and adjustment of his pain management regimen. This includes potential adjustments to his opioid medication, which necessitates clinical oversight to mitigate risks associated with long-term use. His medical regimen also includes medications such as Brilinta and Allopurinol, which require careful management considering his overall health condition and potential interactions.

In summary, the combination of Alfred's medical diagnoses, his significant functional limitations, the required level of skilled care, and the essential nature of the services provided for chronic pain management and physical rehabilitation align with Medicare home health admission guidelines, demonstrating the medical necessity for his admission to home health care.

Homebound Reason (POC Element):

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [ ] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [ ] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Patient requires assistance for safety with ADLs/IADLs.

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadriplegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Experiencing pain impairs/restricts mobility
- [ ] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [ ] Leaving home is exhausting, extended rest periods to recover
- [ ] Extended respiratory recovery after exertion
- [ ] Requires human assist due to unsteady gait/poor balance
- [ ] Weakness and pain from recent surgery
- [ ] Visually impaired-requires constant attendance/assist
- [ ] Cognitively impaired, unable to safely leave home without assist
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Assistance needed for safe mobility and transfer.

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [x] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [ ] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [ ] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

High risk for fall and injury.

Prognosis (POC Element (§484.60 (2.v))):

○ 1-Poor ○ 2-Guarded ● 3-Fair ○ 4-Good ○ 5-Excellent

Functional Limitations (POC Element (§484.60 (2.vi))):

○ 1 - Amputation
○ 2 - Bowel/Bladder (Incontinence)
○ 3 - Contracture
● B- Other (Specify)
○ 4 - Hearing
○ 5 - Paralysis
● 6 - Endurance
● 7 - Ambulation
○ 8 - Speech
○ 9 - Legally blind
○ A - Dyspnea with minimal exertion

Emergency Preparedness (POC Element):

Obtained the patient's emergency contact numbers for the medical record
Discussed patient specific emergency planning options
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted

Acuity Level:

Acuity Level: **Medium Risk or Medium Priority**

Emergency Triage:

Emergency Triage: **Category 2 - Not life threatening but would suffer severe adverse effect**

Additional Emergency Preparedness Information:

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives (POC Element (§484.60 (2.xv))):

Patient has Advance Directives? ● Yes ○ No

If you have checked "yes", fill the following fields:

- [x] Healthcare Representative — Copies to Follow
- [ ] Do Not Resuscitate
- [x] Power of Attorney — Copies to Follow
- [ ] Living Will
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [ ] Full Code
- [ ] The patient has not executed any advance directives to health care.
- [x] The patient was provided with written and verbal information on advance directives.

Comments:
Power of Attorney Additional Info:

Has surrogate? ○ Yes ● No

Surrogate Name:

Surrogate Phone:

Health Screening:

Last Cholesterol Level: mm/dd/yyyy

Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ○ Yes ● No

Last Pap Smear: mm/dd/yyyy

Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy

Last Colonoscopy: mm/dd/yyyy

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control (POC Element (§484.60 (2.i))):

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly

controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

	Onset Date	Severity	
PRIMARY DIAGNOSIS M47.816 - Spondylosis w/o myelopathy or radiculopathy, lumbar region	04/08/2026	0 1 2 **3** 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 1 M17.0 - Bilateral primary osteoarthritis of knee	04/08/2026	0 1 2 **3** 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 2 M75.02 - Adhesive capsulitis of left shoulder	04/08/2026	0 1 2 **3** 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 3 I10. - Essential (primary) hypertension	04/08/2026	0 1 **2** 3 4 ^	**Onset** Exacerbate
OTHER DIAGNOSIS - 4 I25.10 - Athscl heart disease of native coronary artery w/o ang pctrs	04/08/2026	0 1 **2** 3 4 ^	**Onset** Exacerbate
OTHER DIAGNOSIS - 5 E11.9 - Type 2 diabetes mellitus without complications	04/08/2026	0 1 **2** 3 4 ^	**Onset** Exacerbate

(M1028) Active Diagnoses:

Comorbidities and Co-existing Conditions – *Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.*

- [ ] **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- [x] **2. Diabetes Mellitus (DM)**
- [ ] **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up:

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
70

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
215

BMI:
30.85

Comments:

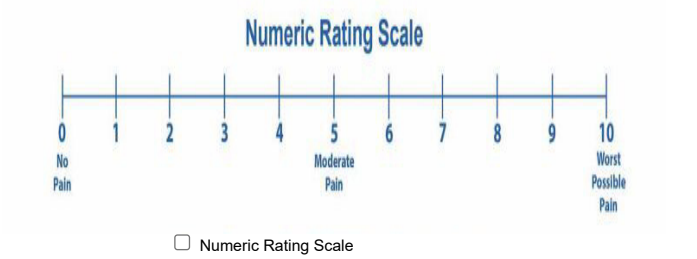
Vital Signs

Temperature: 97.8 Temporal
Respiratory (per minute): 18
Blood Pressure mm/Hg (R): 111 / 86 sitting
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 98
Comments:

Pulse (per minute): Apical = Radial = 94
Weight: 215 reported
Blood Pressure mm/Hg (L): /

O2 Sat in: Room Air

Pain Assessment



5 Collected Using: ☐ Face Scale ☐ Numeric Rating Scale

Pain Location and Description:

1

Location	Start Date	Activity	What makes pain worse?
Bilateral Knee	04/08/2026	At Rest×	Movement×

Intensity	Duration	What makes pain better?	Does pain affect quality of life?
6×	months	Med/Rest×	yes
	Unable to Assess ☐		Unable to Assess ☐

Description	Frequency	Type of Intervention
Sharp×	Frequent×	Opiod Analgesic×

How well it is working	Status
Fairly Well×	COC×

Comments:

(J0510) Pain Effect on Sleep:

Ask patient: *"Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"*

- ○ **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ○ **1. Rarely or not at all**
- ● **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities:

Ask patient: *"Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"*

- ● **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ○ **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities:

Ask patient: *"Over the past 5 days, how often you have limited your day to day activities (excluding rehabilitation therapy session) because of pain?"*

○ **1. Rarely or not at all** ○ **2. Occasionally** ● **3. Frequently** ○ **4. Almost Constantly** ○ **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

Medication (POC Element (§484.60 (2.x))):

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	04/08/2026	Oxycodone	10 mg 10 mg as needed every 4-6 hours	oral	OPIOID ANALGESICS / severe pain	Hold
☐	04/08/2026	Allopurinol	100 mg 100 mg once daily	oral	ANTIGOUT AGENTS / possible gout in right wrist	Old
☐	04/08/2026	ASPI-COR	81 mg DR Tab Once a day	By mouth	NON-OPIOID ANALGESICS /	New
☐	04/08/2026	BRILINTA	60 mg 1 Tab Twice a day	By mouth	PLATELET AGGREGATION INHIBITORS /	New
☐	04/08/2026	Famotidine (Oral Pill)	20 mg 1 Tab Once a day	By mouth	HISTAMINE ANTAGONISTS /	New
☐	04/08/2026	FARXIGA	10 mg 1 Tab Once a day	By mouth	ORAL HYPOGLYCEMIC AGENTS,ORAL /	New
☐	04/08/2026	Fenofibrate (Oral Pill)	54 mg 1 Tab Once a day	By mouth	ANTILIPEMIC AGENTS /	New
☐	04/08/2026	Isosorbide dinitrate (Oral Pill)	30 mg 1 Tab Once a day	By mouth	ANTIANGINALS /	New
☐	04/08/2026	Metoprolol (Oral Pill)	25 mg 1 Tab Twice a day	By mouth	BETA BLOCKERS/RELATED /	New
☐	04/08/2026	MOUNJARO	10 mg/ml Injection 0.5 ml Once a week	Injectable	HYPOGLYCEMIC AGENTS,OTHER /	New
☐	04/08/2026	Nitroglycerin (Sublingual)	0.4 mg 1 Tab see instructions	Sublingual	ANTIANGINALS /	New

	Start Date		Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	04/08/2026	CLEARLAX	17 g Pwdr Once a day	Oral Solution Powder	HYPEROSMOTIC LAXATIVES /	New
☐	04/08/2026	REPATHA	140 mg/ml Auto-Injector 1 ml every 14 days	Injectable	ANTILIPEMIC AGENTS /	New
☐	04/08/2026	Sacubitril/Valsartan (Oral Pill)	24-26 mg 1 Tab Twice a day	By mouth	ANTIHYPERTENSIVE COMBINATIONS /	New

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Allergies (POC Element):

Name	Reaction	Start Date

☐ Add Allergies

None known

(M2001) Drug Regimen Review:

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **0. No – No issues found during review [Go to M2010, Patient/Caregiver High-Risk Drug Education]**
- ○ **1. Yes – Issues found during review**
- ○ **Not assessed/no information**
- ○ **9. NA – Patient is not taking any medications [Go to O0110, Special Treatments, Procedures, and Programs]**

(M2003) Medication Follow-up:

Did the agency contact a physician (or physician designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

○ **0. No** ○ **1. Yes** ○ **Not assessed/no information**

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking 1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication Noted 2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	■
E. Anticoagulant	☐	■
F. Antibiotic	☐	■
H. Opioid	☑	☑
I. Antiplatelet	☑	☑
J. Hypoglycemic (including insulin)	☑	☑
Z. None of the Above	☐	

(M2010) Patient/Caregiver High-Risk Drug Education:

Has the patient/caregiver received instruction on special precautions for all high risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ○ **0. No**
- ● **1. Yes**
- ○ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

(M2020) Management of Oral Medications:

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. ***(NOTE: This refers to ability, not compliance or willingness.)***

- ○ **0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ● **1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ○ **3. Unable to take medication unless administered by another person**
- ○ **NA - No oral medications prescribed**

(M2030) Management of Injectable Medications:

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ○ **0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ● **1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ○ **2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ○ **3. Unable to take injectable medication unless administered by another person**
- ○ **NA - No injectable medications prescribed.**

(O0110) Special Treatments, Procedures, and Programs:

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	■
A2. IV	■

A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations:

Pneumonia:	**Flu:**	**Tetanus:**	**TB:**
☑ Yes ☐ No ☐ Unknown	☑ Yes ☐ No ☐ Unknown	☑ Yes ☐ No ☐ Unknown	☑ Yes ☐ No ☐ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy
TB Exposure:	**Hepatitis B:**	**COVID-19:**	**Additional Immunization:**
☐ Yes ☑ No ☐ Unknown	☑ Yes ☐ No ☐ Unknown	☑ Yes ☐ No ☐ Unknown	☐ Yes ☐ No ☐ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy

Comments:
Pt unable to recall date of immunizations. SN instructed pt on importance of immunizations to prevent communicable illnesses.

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Neuromuscular:
☑ No Problems Identified

Level of Consciousness:
☐ Awake ☐ Alert ☐ Lethargic ☐ Minimally Responsive ☐ Unresponsive ☐ Coma

Impairment:
☐ Speech ☐ Hearing ☐ Visual ☐ Tremors ☐ Numbness ☐ Tingling ☐ Vertigo ☐ Ataxia ☐ Decreased Sensitivity

Speech Impairment (Specify):

Psychosocial (POC Element (§484.60 (2.ii))):
☐ Document as Clinical Narrative

Educational level

Primary Language
English

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language

Learning

◯ Needs interpreter

◯ Mental
◯ Physical

◯ Psychosocial
◯ Functional

Emotions Reported:

◯ Angry
◯ Discouraged
◯ Withdrawn

◯ Flat affect
◯ Difficulty coping
◯ Disorganized

◯ Depressed ◯ Recent ◯ Long Term

Treatment:

Evidence of abuse/neglect/exploitation:

◯ Potential
◯ Verbal/Emotional
◯ Physical

◯ Actual
◯ Financial
◯ No Evidence of Abuse/Neglect/Exploitation

Unable to cope with altered health status, patient:

◯ Lacks motivation ◯ Has unrealistic expectations ◯ Denies problems ◯ Unable to recognize problems

Sleep/Rest

● Adequate ◯ Inadequate

No. of hours slept per night: 7

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☑Yes ☐No ☐NA

Did the patient have a job before admission? ◯Yes ◉No

If yes, do they want to drive post-discharge? ☑Yes ☐No

If yes, do they want to return to work post-discharge? ◯Yes ◯No

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ◯ **Two** ● **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year
- ◯ 0. Missed by > 5 years or no answer
- ◯ 1. Missed by 2-5 years
- ◯ 2. Missed by 1 year
- ● 3. Correct
- ◯ Not assessed/no information

Ask patient: **"What month are we in right now?"**

B) Able to report correct month
- ◯ 0. Missed by > 1 month or no answer
- ◯ 1. Missed by 6 days to 1 month
- ● 2. Accurate within 5 days
- ◯ Not assessed/no information

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week
- ◯ 0. Incorrect or no answer
- ● 1. Correct
- ◯ Not assessed/no information

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"
- ◯ 0. No – could not recall
- ◯ 1. Yes, after cueing ("something to wear")
- ● 2. Yes, no cue required
- ◯ Not assessed/no information

B) Able to recall "blue"
- ◯ 0. No – could not recall
- ◯ 1. Yes, after cueing ("a color")
- ● 2. Yes, no cue required
- ◯ Not assessed/no information

C) Able to recall "bed"
- ◯ 0. No – could not recall
- ◯ 1. Yes, after cueing ("a piece of furniture")
- ● 2. Yes, no cue required
- ◯ Not assessed/no information

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** 15

(C1310) Signs and Symptoms of Delirium (from CAM©):

A. Acute Onset of Mental Status Change:

Is there evidence of an acute change in mental status from the patient's baseline?

● 0. No ◯ 1. Yes

B. Inattention:

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

● 0. Behavior not present
◯ 1. Behavior continuously present, does not fluctuate
◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
◯ Not assessed/no information

C. Disorganized thinking:

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

● 0. Behavior not present
◯ 1. Behavior continuously present, does not fluctuate
◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
◯ Not assessed/no information

D. Altered level of consciousness:

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- ***vigilant*** *– startled easily to any sound or touch* *lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch*
- ***stuporous*** *– very difficult to arouse and keep aroused for the interview*
- ***comatose*** *– could not be arouse*

● 0. Behavior not present
◯ 1. Behavior continuously present, does not fluctuate
◯ 2. Behavior present, fluctuates (comes and goes, changes in severity)
◯ Not assessed/no information

(D0150) Patient Mood Interview (PHQ-2 to 9):

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	No ×	Never or 1 day
B. Feeling down, depressed, or hopeless	No ×	Never or 1 day
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

(D0700) Social Isolation:

How often do you feel lonely or isolated from those around you?

- [x] **0. Never**
- [ ] **1. Rarely**
- [ ] **2. Sometimes**
- [ ] **3. Often**
- [ ] **4. Always**
- [ ] **7. Patient declines to respond**
- [ ] **8. Patient unable to respond**

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed):

(Mark all that apply)

- [ ] **1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required**
- [ ] **2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions**
- [ ] **3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.**
- [ ] **4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)**
- [ ] **5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)**
- [ ] **6. Delusional, hallucinatory, or paranoid behavior**
- [x] **7. None of the above behaviors demonstrated**

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- [x] **0. Never**
- [ ] **1. Less than once a month**
- [ ] **2. Once a month**
- [ ] **3. Several times each month**
- [ ] **4. Several times a week**
- [ ] **5. At least daily**

(M1700) Cognitive Functioning:

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- [x] **0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.**
- [ ] **1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.**
- [ ] **2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility**
- [ ] **3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.**
- [ ] **4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.**

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- [x] **0. Never**
- [ ] **1. In new or complex situations only**
- [ ] **2. On awakening or at night only**
- [ ] **3. During the day and evening, but not constantly**
- [ ] **4. Constantly**
- [ ] **5. Patient nonresponsive**

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- [x] **0. None of the time**
- [ ] **1. Less often than daily**
- [ ] **2. Daily, but not constantly**
- [ ] **3. All of the time**
- [ ] **NA - Patient nonresponsive**

Mental Status (POC Element (§484.60 (2.ii)):

- [x] 1 - Oriented
- [ ] 2 - Comatose
- [ ] 3 - Forgetful
- [ ] 4 - Depressed
- [ ] 5 - Disoriented
- [ ] 6 - Lethargic
- [ ] 7 - Agitated
- [x] 8 - Alert
- [ ] 9 - Confused
- [ ] 10 - Dementia
- [ ] 11 - Assaultive
- [ ] 12 - Delirious
- [ ] 13 - Wanders
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is Alert and Oriented X3, dressed appropriately and able to make needs known.

Has there been a sudden/acute change in their mental status? [ ] Yes [x] No Reported by: [ ] If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? [ ] No [ ] Yes

Comments:

Cultural:

Is religion important to the patient? [ ] Yes [x] No

Patient's primary source of emotional support:
Wife

Eyes and Ears

(B1000) Vision:

Ability to see in adequate light *(with glasses or other visual appliances)*

- [ ] 0. Adequate – sees fine detail, such as regular print in newspapers/books
- [x] 1. Impaired – sees large print, but not regular print in newspapers/books
- [ ] 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- [ ] 3. Highly impaired – object identification in question, but eyes appear to follow objects
- [ ] 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision Assessment:

- [ ] No Problems Identified
- [x] Glasses
- [ ] Contact Lenses: [ ] Right [ ] Left
- [ ] Glaucoma: [ ] Right [ ] Left
- [ ] Blurred Vision: [ ] Right [ ] Left
- [ ] Macular Degeneration: [ ] Right [ ] Left
- [ ] Blindness: [ ] Right [ ] Left
- [ ] Cataracts: [ ] Right [ ] Left
- [ ] Other (Specify):

Pupils:

- [x] PERRLA
- [ ] Anisocoria: [ ] Right [ ] Left
- [ ] Other (Specify):

Comments:

(B0200) Hearing:

Ability to hear *(with hearing aid or hearing appliances if normally used)*

- [x] 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- [ ] 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- [ ] 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- [ ] 3. Highly impaired – absence of useful hearing

Hearing Assessment:

- [x] No Problems Identified
- [ ] Right Hearing Impairment
- [ ] Left Hearing Impairment
- [ ] Deaf
- [ ] Hearing Aids: [ ] Right [ ] Left

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds:

- [ ] No Problems Identified
- [ ] Irregular: [ ] Known Hx [ ] New Finding
- [ ] Gallops: [ ] Known Hx [ ] New Finding
- [ ] Clicks: [ ] Known Hx [ ] New Finding
- [ ] Murmur: [ ] Known Hx [ ] New Finding
- [ ] FrictionRub: [ ] Known Hx [ ] New Finding

Comments:
PMH of 2 heart attacks, age 35 & 37

Edema:

- [ ] Right Upper Extremities:
- [ ] Left Upper Extremities:
- [ ] Right Lower Extremities:
- [ ] Left Lower Extremities:

Others:

Others:

Comments:

Peripheral Pulses:

Radial: [x] Strong [ ] Bounding [ ] Weak [ ] Thready [ ] Absent

Pedal: [x] Strong [ ] Bounding [ ] Weak [ ] Thready [ ] Absent

Fluid Imbalance and Systemic Distress:

- [ ] Cramps
- [ ] Claudication
- [ ] Fatigue
- [ ] Face
- [ ] Neck
- [ ] Abdomen

Capillary Refill:

- [x] < 3 secs
- [ ] > 3 secs

Chest/Thorax

Chest Pain:

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Nausea
- [ ] Other (Specify):

Associated with:

- [ ] Shortness of Breath
- [ ] Activity
- [ ] Rest
- [ ] Diaphoresis
- [ ] Frequency/Duration:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- ( ) 0. Patient is not short of breath
- (x) 1. When walking more than 20 feet, climbing stairs
- ( ) 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- ( ) 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- ( ) 4. At rest (during day or night)

Respiratory Status:

Respiratory Distress Indicators:

- [ ] Dyspnea:
- [ ] At Rest
- [ ] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor
- [ ] O2 Liters per minute:
- [ ] O2 Saturation (%):

Frequency/Timing of Oxygen Use:

- [ ] PRN
- [ ] Continuous
- [ ] Only at bedtime
- [ ] Other (Specify):

Route/Delivery:

- [ ] Nasal Cannula
- [ ] Trach Cuff
- [ ] Mask (Specify):
- [ ] Other (Specify):

Lung Sounds:

Anterior Right:
Anterior Right: **Clear**

Anterior Left:
Anterior Left: **Clear**

Posterior Upper Right:
Posterior Upper Right: **Clear**

Posterior Upper Left:
Posterior Upper Left: **Clear**

Posterior Lower Right:
Posterior Lower Right: **Clear**

Posterior Lower Left:
Posterior Lower Left: **Clear**

Comments:

Cough:

Cough Characteristics:

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic

Productivity of Cough:

- [ ] Non-Productive
- [ ] Productive:

Sputum Characteristics (if Productive):

- [ ] Brown
- [ ] Clear
- [ ] Cloudy
- [ ] Bloody
- [ ] Yellow
- [ ] White
- [ ] Hemoptysis:
- [ ] Amount (Specify):
- [ ] Frequency (Specify):

GASTROINTESTINAL & GENITOURINARY

Gastrointestinal (Mouth - Anus)

Mouth and Throat (Assess oral health and swallowing ability):

- [ ] Dentures:
- [ ] Upper
- [ ] Lower
- [ ] Partial
- [ ] Bleeding Gums/Gingivitis
- [ ] Toothache
- [ ] Ulcerations
- [ ] Lesion
- [ ] Dysphagia
- [ ] Hoarseness
- [x] Sore Throat
- [ ] Masses/Tumors

Comments:
Pt reports losing 1 tooth/year since moving to Las Vegas from HI

(M1870) Feeding or Eating:

*Current ability to feed self meals and snacks safely. Note: This refers only to the process of **eating**, **chewing**, and **swallowing**, not preparing the food to be eaten.*

- ( ) 0. Able to independently feed self.
- (x) 1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- ( ) 2. Unable to feed self and must be assisted or supervised throughout the meal/snack.
- ( ) 3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- ( ) 4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- ( ) 5. Unable to take in nutrients orally or by tube feeding.

Appetite (%):

Appetite Percentage of (Meals per day):

Snacks (Per day):

Appetite:

- [ ] Increase
- [ ] Decreased
- [x] Unchanged

Intake:

- [x] Adequate
- [ ] Inadequate

Nutritional Abnormalities:

- [ ] Anoxeria
- [ ] Cachexia
- [ ] Decreased Appetite
- [ ] Constipation

(K0520) Nutritional Approaches:

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	[ ]
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	[ ]
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	[ ]
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	[x]
Z. None of the above	[ ]

#16 Nutritional Requirements (POC Element (§484.60 (2.ix))):

- [ ] NPO
- [ ] No Added Salt
- [ ] Low Salt
- [ ] Low Fat
- [ ] Low Cholesterol
- [x] No Concentrated Sweets
- [ ] Controlled Carbohydrates
- [ ] Regular
- [ ] Fluid Restriction:

Other:

Heart healthy diet.

Nutritional Health Screen	Score	Nutritional Health Screen	Score
◯ Without reason, has lost more than 10 lbs in the last 3 months	15	◯ Does not always have enough money to buy foods need	10
● Has an illness or condition that made pt change the type and/or amount of food eaten	10	◯ Eats alone most of the time	5
◯ Has open decubitus, ulcer, burn or wound	10		
◯ Eats fewer than 2 meals a day	10	● Takes 3 or more prescribed or OTC medications a day	5
◯ Has a tooth/mouth problem that makes it hard to eat	10	◯ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
◯ Has 3 or more drinks of beer, liquor or wine almost every day	10	◯ Frequently has a diarrhea or constipation	5
		Total	15

● Good Nutritional Status (Score 0 - 25) ◯ Moderate Nutritional Status (Score 25 - 54) ◯ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:
Instructed patient to follow adequate diet and fluid intake to ensure proper nutrition and hydration.

Weight:

☐ No Change ☐ Weight Gain ☑ Weight Loss

Comments:
Pt has lost 115 pounds in last year with medication, diet and exercise

(M1620) Bowel Incontinence Frequency:

● 0. Very rarely or never has bowel incontinence
◯ 1. Less than once weekly
◯ 2. One to three times weekly
◯ 3. Four to six times weekly
◯ 4. On a daily basis
◯ 5. More often than once daily
◯ NA - Patient has ostomy for bowel elimination
◯ UK - Unknown

(M1630) Ostomy for Bowel Elimination:

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

● 0. Patient does not have an ostomy for bowel elimination
◯ 1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
◯ 2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

Bowel Sounds (Abdominal Quadrants):

Right Lower:	Right Upper:	Left Lower:	Left Upper:	Last Bowel Movement:
Right Lower: **Normoactive**	Right Upper: **Normoactive**	Left Lower: **Normoactive**	Left Upper: **Normoactive**	04/07/2026

GI Symptoms:

☐ Nausea ☐ Vomiting ☐ Reflux/Indigestion ☐ Bowel Incontinence ☐ Diarrhea ☐ Constipation

Abdominal Assessment:

☑ Abdominal Soft/Non-tender ☐ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm):

Hemorrhoids:

☐ External Hemorrhoids ☐ Internal Hemorrhoids ☐ Laxative/Enema Use

Stool Appearance:

☐ Gray ☐ Tarry ☑ Brown/Tan ☐ Black ☐ Fresh Blood ☐ Other (Specify):

Ostomy/Stoma (if applicable):

☐ Moist ☐ Dry ☐ Pinkished-Red ☐ Purple ☐ Gray ☐ Black ☐ Odor

Peristomal Skin Characteristics:

☐ Clean ☐ Healthy ☐ Red ☐ Weeping ☐ Peeling ☐ Inflamed ☐ Other (Specify):

Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

● 0. No ◯ 1. Yes ◯ NA - Patient on prophylactic treatment ◯ UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

◯ Patient is incontinent
- ◯ Day time
- ◯ Night time
- ◯ Day and Night
- ◯ Timed-voiding defers incontinence
- ◯ Occasional stress incontinence

◯ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

● No incontinence or catheter (includes anuria or ostomy for urinary drainage)

Genitourinary Assessment:

☑ No Problems Identified

Abnormalities:

☐ Frequency ☐ Dysuria ☐ Urgency ☐ Anuria ☐ Nocturia ☐ Oliguria
☐ Retention ☐ Hematuria ☐ Irrigation ☐ Incontinence ☐ Diapers Used ☐ Penile Discharge
☐ Vaginal Bleeding/Discharge ☐ Other (Specify):

Urine Color:

☐ Transparent ☑ Pale Yellow ☐ Brownish Orange ☐ Pinkish Red

☐ Dark Yellow ☐ Blue or Green ☐ Foamy ☐ Transparent Yellow

Urine Consistency:

☑ Clear ☐ Sediment ☐ Odorous ☐ Cloudy

Urinary Catheter:

☐ Yes ☑ No

☐ External Catheter ☐ Foley ☐ Suprapubic ☐ Nephrostomy

☐ Last Catheter Change: mm/dd/yyyy ☐ Fr: ☐ cc:

Comments:

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- ● 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
- ○ 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage:

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non┐removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persis tent blue or purple hues.

Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- ● 0. No **[GO TO M1340]**
- ○ 1. Yes, patient has BOTH observable and unobservable stasis ulcers
- ○ 2. Yes, patient has observable stasis ulcers ONLY
- ○ 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non┐removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

○ 1. One ○ 2. Two ○ 3. Three ○ 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable:

○ 1. Fully granulating ○ 2. Early/partial granulation ○ 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- ● 0. No
- ○ 1. Yes, patient has at least one observable surgical wound
- ○ 2. Surgical wound known but not observable due to non-removable dressing/device

(M1342) Status of Most Problematic Surgical Wound that is Observable:

○ 0. Newly epithelialized ○ 1. Fully granulating ○ 2. Early/partial granulation ○ 3. Not healing

Skin Color:

☑ Normal ☐ Pale ☐ Jaundice ☐ Cyanotic

Skin Turgor:

☐ Good ☑ Fair ☐ Poor

Skin Abnormalities:

☑ Dry ☐ Warm ☐ Cool ☐ Rash ☐ Ulcer ☐ Ostomy ☐ Incision ☐ Diaphoretic ☐ Other (Specify):

Comments:

Norton Scale for Pressure Ulcer Risk Assessment:

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	○ Good	● Fair	○ Poor	○ Very Bad
Mental Condition	● Alert	○ Apathetic	○ Confused	○ Stuporous
Activity	● Ambulant	○ Walks With Help	○ Chairbound	○ Bedfast
Mobility	○ Full	● Slightly Impaired	○ Very Limited	○ Immobile
Incontinence	● None	○ Occassional	○ Usually Urinary	○ Urinary and Fecal
			Total Score:	18

Wound

Wound Worksheet:

Fetch Wounds From Previous Wound Note

SAFETY & RISK ASSESSMENT

Home Environment/Home Safety Assessment

- [ ] Architectural Barriers
- [ ] No fire extinguisher
- [ ] No emergency exit plan
- [ ] Medications are improperly stored
- [ ] Inadequate electricity/cooling/heating
- [ ] Rodent/insect infestation
- [ ] No telephone
- [ ] Inadequate supply of water
- [ ] Unsafe electrical wiring
- [ ] Cluttered surroundings
- [ ] Inadequate sanitation/plumbing
- [ ] Nonworking stove/refrigerator
- [ ] Inadequate railing
- [ ] Inadequate smoke detectors on all levels of home
- [ ] Exposed flammables
- [ ] Unsafe oxygen use
- [ ] Unsafe storage of equipments
- [ ] Inadequate Lighting

Others (Specify):
No Home Safety Risk Noted.

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

ADL (Bathing, dressing, toileting, eating/feeding, bowel):
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances):
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Psychosocial Support:
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Assistance with doctors appointment, delivery of medications:
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Management of Finances:
- [x] Family/Friends
- [ ] Provider Services
- [ ] Paid Caregiver
- [ ] Volunteer Organizations

Comments:

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	○ Patient Lives alone around the clock	○ Patient Lives alone Regular daytime	○ Patient Lives alone Regular Night time	○ Patient Lives alone Short-term Assistance	○ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	● Patient lives with other person(s) in the home Around the clock	○ Patient lives with other person(s) in the home Regular daytime	○ Patient lives with other person(s) in the home Regular Night time	○ Patient lives with other person(s) in the home Short-term Assistance	○ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	○ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	○ Regular daytime	○ Patient lives in congregate situation Regular Night time	○ Patient lives in congregate situation Short-term Assistance	○ Patient lives in congregate situation No Assistance available

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety *(for example, due to cognitive impairment)*

- ○ 0. No assistance needed – patient is independent or does not have needs in this area
- ○ 1. Non-agency caregiver(s) currently provide assistance
- ● 2. Non-agency caregiver(s) need training/supportive services to provide assistance
- ○ 3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ○ 4. Assistance needed, but no non-agency caregiver(s) available

Safety Measures (POC Element (§484.60 (2.xi))):

- ○ 1 - Bleeding precautions
- ○ 2 - O_2 precautions
- ○ 3 - Seizure precautions
- ● 4 - Fall precautions
- ○ 5 - Aspiration precautions
- ○ 6 - Siderails up
- ○ 7 - Elevate head of bed
- ○ 8 - 24 hr. supervision
- ● 9- Clear Pathways up
- ○ 10 - Lock w/c with transfers
- ● 11 - Infection control measures
- ○ 12 - Walker/cane
- ○ 13 - Sharps Precaution
- ○ 14 - Other:

Hospitalization Risk Assessment

(M1033) Risk for Hospitalization:

Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- [ ] 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- [ ] 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- [ ] 3. Multiple hospitalizations (2 or more) in the past 6 months
- [ ] 4. Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5. Decline in mental, emotional, or behavioral status in the past 3 months
- [ ] 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7. Currently taking 5 or more medications
- [x] 8. Currently reports exhaustion
- [x] 9. Other risk(s) not listed in 1 - 8
- [ ] 10. None of the above

Additional Risk Information:
HTN,DM 2

Fall Risk Test

- [ ] N/A
- [x] MAHC 10* - Fall Risk Assessment Tool
- [ ] TUG Assessment Tool

MAHC 10* - FALL RISK ASSESSMENT TOOL

Age 65+: ○ Yes ● No

Diagnosis (3 or more co-existing): ● Yes ○ No

- *Includes only documented medical diagnosis*

Prior history of falls within 3 months: ○ Yes ● No
- *An unintentional change in position resulting in coming to rest on the ground or at a lower level*

Incontinence: ○ Yes ● No
- *Inability to make it to the bathroom or commode in timely manner.*
- *Includes frequency, urgency, and/or nocturia.*

Visual impairment: ● Yes ○ No
- *Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tol erance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.*

Impaired functional mobility: ● Yes ○ No
- *May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, imp aired coordination or improper use of assistive devices.*

Environmental hazards: ○ Yes ● No
- *May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.*

Poly Pharmacy (4 or more prescriptions-any type): ● Yes ○ No
- *All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquili zers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.*

Pain affecting level of function: ● Yes ○ No
- *Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.*

Cognitive impairment: ○ Yes ● No
- *Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsi vity, memory deficits. Consider patients ability to adhere to the plan of care.*

A score of 4 or more is considered at risk for falling: 5.00

Fire Assessment for Patient with Oxygen

☑ **Patient is not using oxygen**

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities:

Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.
- ***3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper*
- ***2. Needed Some Help** – Patient needed partial assistance from another person to complete activities*
- ***1. Dependent** – A helper completed the activities for the patient.*
- ***8. Unknown***
- ***9. Not Applicable***

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓	○	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	✓	○	○	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	✓	○	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	✓	○	○	○	○

(GG0110) Prior Device Use:

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (***Check all that apply***)

- ☐ **A. Manual wheelchair**
- ☐ **B. Motorized wheelchair and/or scooter**
- ☐ **C. Mechanical lift**
- ☐ **D. Walker**
- ☐ **E. Orthotics/Prosthetics**
- ☑ **Z. None of the above**

(GG0130) Self Care

(GG0130) SOC/ROC Performance:

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance –
Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130A) A. Eating:
- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130B) B. Oral Hygiene:
- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130C) C. Toileting Hygiene:
- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance

(GG0130E) E. Shower/Bathe Self:
- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance

- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130F) F. Upper Body Dressing:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130H) H. Putting on/Taking Off Footwear:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130G) G. Lower Body Dressing:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ● 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(M1800) Grooming:

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- ○ 0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.
- ● 1. Grooming utensils must be placed within reach before able to complete grooming activities.
- ○ 2. Someone must assist the patient to groom self.
- ○ 3. Patient depends entirely upon someone else for grooming needs

(M1810) Ability to Dress Upper Body:

*Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:*

- ○ 0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.
- ○ 1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.
- ● 2. Someone must help the patient put on upper body clothing.
- ○ 3. Patient depends entirely upon another person to dress the upper body.

(M1820) Ability to Dress Lower Body:

*Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:*

- ○ 0. Able to obtain, put on, and remove clothing and shoes without assistance.
- ○ 1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.
- ● 2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.
- ○ 3. Patient depends entirely upon another person to dress lower body.

(M1830) Bathing:

*Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).***

- ○ 0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- ○ 1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.
- ● 2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- ○ 3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- ○ 4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- ○ 5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- ○ 6. Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ○ 0. Able to get to and from the toilet and transfer independently with or without a device
- ● 1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- ○ 2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- ○ 3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently
- ○ 4. Is totally dependent in toileting.

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ○ 0. Able to manage toileting hygiene and clothing management without assistance.
- ● 1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- ○ 2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- ○ 3. Patient depends entirely upon another person to maintain toileting hygiene

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ○ 0. Able to independently transfer.
- ● 1. Able to transfer with minimal human assistance or with use of an assistive device.
- ○ 2. Able to bear weight and pivot during the transfer process but unable to transfer self.
- ○ 3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person
- ○ 4. Bedfast, unable to transfer but is able to turn and position self in bed.
- ○ 5. Bedfast, unable to transfer and is unable to turn and position self

(M1860) Ambulation/Locomotion:

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ○ 0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
- ○ 1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.
- ○ 2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- ● 3. Able to walk only with the supervision or assistance of another person at all times.
- ○ 4. Chairfast, unable to ambulate but is able to wheel self independently.

- ◯ 5. Chairfast, unable to ambulate and is unable to wheel self
- ◯ 6. Bedfast, unable to ambulate or be up in a chair.

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right:

The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170B) B. Sit to lying:

The ability to move from sitting on side of bed to lying flat on the bed.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed:

The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170D) D. Sit to stand:

The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer:

The ability to transfer to and from a bed to a chair (or wheelchair).

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170F) F. Toilet transfer:

The ability to get on and off a toilet or commode.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170G) G. Car transfer:

The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170I) I. Walk 10 feet:

Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns:

Once standing, the ability to walk 50 feet and make two turns.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170K) K. Walk 150 feet:

Once standing, the ability to walk at least 150 feet in a corridor or similar space.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces:

The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170M) M. 1 step (curb):

The ability to go up and down a curb and/or up and down one step. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170N) N. 4 steps:

The ability to go up and down four steps with or without a rail. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170O) O. 12 steps:

The ability to go up and down 12 steps with or without a rail.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170P) P. Picking up object:

The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

● 0. No ○ 1. Yes ○ (-) Not assessed/no information

(GG0170R) R. Wheel 50 feet with two turns:

Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

○ 1. Manual ○ 2. Motorized ○ (-) Not assessed/no information

(GG0170S) S. Wheel 150 feet:

Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

○ 1. Manual ○ 2. Motorized ○ (-) Not assessed/no information

Gait and Mobility Abnormalities:

- ☐ No Problems Identified
- ☑ Weakness
- ☐ Poor Balance
- ☐ Bed Bound
- ☐ Chair Bound
- ☐ Joint Stiffness
- ☐ Shuffling/Wide-based Gait
- ☐ Orthopedic Aftercare
- ☐ High Risk for Falls
- ☐ Weight Bearing Restriction
- ☑ Poor Conditioning
- ☐ Fracture:
- ☐ Joints Pain:
- ☐ Amputation:
- ☐ Atrophy:
- ☐ Paresthesia:
- ☐ Contractures (Site):
- ☐ Foot Drop: ☐ Right ☐ Left

Fall(s) since last visit (Specify):

Weakness (Specify):

Change in ADL (Explain):

Comments:

Range of Motion:

- ☐ Limited Range of Motion: ☐ RUE ☐ RLE ☐ LUE ☐ LLE
- ☐ Quadriplegia
- ☐ Hemiplegia
- ☐ Paraplegia

Comments:

Grip Strength:

☑ Normal ☐ Strong ☐ Weak ☑ Equal ☐ Unequal ☐ Bilateral ☐ Right ☐ Left

Others (Specify):

Comments:

DME and Supplies (POC Element (§484.60 (2.iii))):

DME Lookup

#18B Activities Permitted (POC Element (§484.60 (2.viii))):

- ○ 1- Complete bedrest
- ○ 2 - Bedrest/BRP
- ● 3 - Up as tolerated
- ● 4 - Transfer bed/chair
- ● 5 - Exercises prescribed
- ○ 6 - Partial weight bearing
- ○ 7 - Independent in home
- ○ 8 - Crutches
- ○ 9 - Cane
- ○ A - Wheelchair
- ○ B - Walker
- ○ C - No restrictions
- ○ D - Other (specify)

Rehabilitation Potential (§484.60 (2.vi))):

- ○ Poor
- ● Fair
- ○ Good
- ○ Excellent

Others

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic? ☑ Yes ☐ No **If Yes, How Long?**

Diabetes Type: ☐ Type 1 Diabetes Mellitus (T1DM) ☑ Type 2 Diabetes Mellitus (T2DM)

Insulin Administered By: ☐ Patient ☐ Caregivier ☐ SN

Glucose Level Issue: ☐ Hypoglycemia (Low Blood Sugar) ☐ Hyperglycemia (High Blood Sugar)

Frequency of Blood Sugar Issues: ☑ Daily ☐ Weekly ☐ Monthly ☐ Rarely ☐ As needed

Does patient or caregiver routinely perform inspection of the patient's lower extremities? ☑ Yes ☐ No ☐ N/A

Symptoms & Observations:

- ☐ Fatigue (related to blood sugar)
- ☐ Shakiness/Tremors
- ☐ Sweating
- ☐ Confusion/Irritability
- ☐ Hunger
- ☐ Dizziness
- ☐ Blurred Vision
- ☐ Dry Mouth/Excessive Thirst
- ☐ Frequent Urination
- ☑ Weight Loss (unintentional)
- ☐ Weight Gain (unintentional)

Specify Other Symptoms or Patterns:
Pt reports A1C of 6 during last PCP visit. Denies regularly checking BG at home.

PLAN OF CARE ORDERS AND DISCHARGE PLAN

Plan of Care Orders

(POC Element):

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1x/wk x 1 wk ; evaluation only

MSW Frequency:

PT Frequency: evaluation and treatment effective on 04/11/2026

OT Frequency:

HHA Frequency:

ST Frequency

Dietitian Frequency:

*****MD SIGNATURE COVERS ALL PAGES*****
ADMIT PATIENT TO A PLUS HOME HEALTH SYSTEMS LLC SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

The patient meets the criteria for homebound status due to illness and the medically contraindicated nature of leaving his home. He experiences significant mobility limitations, including an inability to tolerate standing or walking for extended periods due to pain. Leaving home is described as a taxing effort, with severe pain during ambulation, requiring considerable rest and recovery afterward. These limitations contribute to the patient's prognosis of functional limitations in ambulation and endurance.

Frequency and duration:
Skilled Nursing Visit Frequency 1 time a week time 1 week; evaluation for skilled observation and assessment, medication review and reconciliation, pain management, wound assessment and wound care infection control measures, compliance with treatment regimen and therapeutic diet, home safety and disease process teaching.
Physical Therapy Visit Frequency Evaluation and treatment for mobility, gait, transfer, established therapeutic exercises, strengthening, home safety and fall prevention techniques training.
Patient Goal:
The patient seeks to enhance overall health, functional capacity, and independence, while improving knowledge and application of self-care management. The patient also aims to prevent signs and symptoms of disease exacerbation, reduce the risk of falls and injuries, and achieve decreased pain levels in the lower back and BLE to improve mobility, comfort, and ability to perform daily activities.

Skilled Intervention

Assessment/Instruction/Performance:
Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment of all body systems with OASIS elements evaluation was initiated. Reconciled facility discharge medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed the plan of care draft with the patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

SN assessed the patient's lower back and bilateral lower extremity (BLE) pain, noting intensity, location, and aggravating or relieving factors, while also evaluating mobility, gait, and activity tolerance. SN monitored for any signs of worsening symptoms related to lumbar spondylosis and checked for any neurological changes. SN provided education on proper body mechanics, posture, and safe movement techniques to help reduce strain on the lower back during daily activities. SN encouraged the use of prescribed pain management strategies, including taking medications as ordered, and reinforced non-pharmacologic methods such as proper positioning, use of heat or cold packs, and gentle stretching. SN guided the patient through basic range-of-motion and strengthening exercises to support the lumbar area as tolerated. SN also reinforced fall prevention strategies, assessed the environment for safety concerns, and encouraged pacing of activities with adequate rest to help improve comfort, mobility, and overall function.

SN assessed the patient's respiratory status, noting shortness of breath with minimal exertion and evaluating breathing pattern, lung sounds, oxygen saturation, and activity tolerance. SN monitored

for signs of respiratory distress and encouraged reporting of any worsening symptoms. Education was provided on energy conservation, including pacing, rest breaks, and avoiding overexertion. SN instructed on breathing techniques such as pursed-lip and diaphragmatic breathing to improve oxygenation. Proper use of medications and oxygen therapy was reinforced. The patient was guided to gradually increase activity as tolerated, with emphasis on safety, fall prevention, and seeking assistance when short of breath.

SN assessed the patient's sore throat, including onset, severity, associated symptoms, and vital signs, while observing for infection. SN monitored for worsening symptoms and encouraged fluids and rest. Education was provided on comfort measures such as warm saline gargles and avoiding irritants. SN reinforced proper medication use and emphasized hand hygiene and infection prevention.

SN assessed the patient's lower back and BLE pain rated 6/10, including location, intensity, and aggravating/relieving factors, and evaluated mobility and gait. SN monitored for worsening symptoms and neurological changes related to lumbar spondylosis. Teaching was provided on proper body mechanics, posture, and safe movement. SN reinforced use of prescribed pain medications and non-pharmacologic measures such as positioning, heat/cold therapy, and gentle stretching. SN encouraged light ROM exercises as tolerated and emphasized fall prevention, activity pacing, and safety to improve comfort and mobility.

SN assessed fall risk by evaluating gait, balance, strength, mobility, safety awareness, and history of falls, including medication effects. SN monitored ambulation for instability. Education was provided on fall prevention, including assistive device use, non-skid footwear, and keeping items within reach. SN assessed the environment for hazards and reinforced maintaining a safe home setting. SN encouraged slow position changes, safe transfers, asking for assistance, and regular activity as tolerated to maintain strength and reduce fall risk.

Re-checked pain post SOC Visit reduced to 2/10 from 6/10. SN provided patient with rest period and comfort during the visit.

Patient verbalized 75% of understanding on the disease process taught during this visit. Patient able to teach back 3 out of 4 non-pharmacologic pain-relieving measure, dyspnea relieving measures and fall prevention measures. Patient also redemonstrated 80% of home safety and fall prevention measures without the use of assistive device as per patient still demonstrates unsteady gait. CG verbalized 75% of understanding and is available to assist patient for safety with ADLs/IADLs, medication and diet compliance for the patient.

Conference With:

☐ **MD** ☑ **SN** ☐ **PT** ☐ **OT** ☐ **ST** ☐ **MSW** ☐ **HHA** ☑ **DON/DOPCS** ☑ **Clinical Coordinator** ☐ **Clinical Manager**

Name:
Kristine Macaraeg -RN CM

Regarding:
SOC, POC, Visit frequencies and update of patient health condition.

Physician Contacted Re:
SOC, POC, Visit frequencies and update of patient health condition.

Order Changes:

Plan for Next Visit:
SN eval only; PT to eval/treat

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge (POC Element (§484.60 (2.xiii))):

SN instructed patient to take pain medication before pain becomes severe to achieve better pain control. Always refill your meds before you run out of them. SN instructed patient on nonpharmacologic pain relief measures, including relaxation techniques, positioning, ice, and rest. SN instructed to report to physician if experiencing unacceptable pain level, pain level greater than 6/10, pain medications not effective, unable to tolerate pain medications, or pain affecting ability to perform normal activities.

SN instructed pt to keep appointments as scheduled. SN Evaluation only.

Discharge Plans

(POC Element):

Able to understand medication regime and care related to diagnoses.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Discharge patient to self-care.
Discharge to caregiver.
Able to return to an independent level of care.
When goals are met.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

☑ **Patient/Family** ☐ **Care Manager** ☐ **Physician** ☐ **Other (specify):**

Discharge Plan:

☑ **To self-care when goals met** ☐ **To self care when max potential achieved** ☐ **To outpatient therapy with MD approval**

☐ **Others (Specify):**

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

(POC Element):

Alfred S. Tuia is a 47-year-old Hawaiian male. He has been referred for home health care on March 23, 2026, and the start of care date is set for April 8, 2026. His attending physician is Dr. Carlos Nunez Cendan. The referral to home health services was initiated due to the patient's need for ongoing management and support related to his health conditions. It is notable that the patient has not been discharged from any inpatient facility within the past 14 days.

The patient meets the criteria for homebound status due to illness and the medically contraindicated nature of leaving his home. He experiences significant mobility limitations, including an inability to tolerate standing or walking for extended periods due to pain. Leaving home is described as a taxing effort, with severe pain during ambulation, requiring considerable rest and recovery afterward. These limitations contribute to the patient's prognosis of functional limitations in ambulation and endurance.

Alfred's primary diagnosis is compounded by comorbidities, most notably diabetes mellitus. His overall health assessment indicates that he has a medium risk acuity level, classified as Category 2 for emergency triage, meaning he is not in life-threatening condition but could face severe adverse effects without appropriate care.

In terms of a head-to-toe assessment, the patient's cardiopulmonary status reveals a pulse of 94 beats per minute, blood pressure at 111/86 mmHg, and a respiratory rate of 18 breaths per minute. He has a PMH of 2 heart attacks at ages 35 and 37. Gastrointestinal and integumentary assessments show no significant abnormalities. The patient's integumentary system is intact, with no evidence of pressure ulcers or wounds.

Psychosocially, Alfred presents as alert and oriented, with no signs of confusion or disorganized thinking. His mood is stable, scoring a total severity score of 0 on the PHQ-2, indicating he does not feel lonely or isolated. His cognitive function is intact, and he demonstrates the ability to understand and follow directions without difficulty.

Vital signs collected during the assessment show a temperature of 97.8°F, which falls within the normal range. His living arrangements indicate that he resides with others, yet he does not require assistance in this area. In terms of activities of daily living (ADLs), Alfred is largely independent. He can perform grooming, dressing, bathing, and toileting without assistance, although he does require support for maintaining toileting hygiene.

Alfred's mobility is assessed as independent, allowing him to walk on various surfaces and negotiate stairs without help or aids. Severe pain restricts his mobility. His functional ability is categorized as independent in self-care and indoor ambulation. The patient engages in activities such as exercise, which supports his functional capacity and overall well-being.

Socially, Alfred has access to emergency contact numbers and has developed an individualized emergency preparedness plan in collaboration with the home health team. His support system

includes family members or friends who are available to assist him if needed.

A review of Alfred's current medications indicates he is taking five or more medications, which raises the potential risk for adverse effects and drug interactions. He is on high-risk medications, including opioids, antiplatelet agents, and hypoglycemic agents (including insulin). The patient and his caregiver have received instructions regarding the precautions necessary for managing these medications, which he is able to take independently at the correct times and dosages.

In terms of nutrition, Alfred is following a therapeutic diet that includes low salt, low cholesterol, and a restriction on concentrated sweets. His nutritional health screening indicates a good nutritional status, with a score that suggests he has made dietary adjustments due to his health conditions.

In conclusion, there is a clear medical necessity for skilled services in home health care for Alfred S. Tuia. His complex medical needs, including hypertension and diabetes management, along with his homebound status and functional limitations, warrant regular, skilled nursing visits. A visit frequency of two to three times a week is recommended to ensure continuous monitoring, medication management, and support in achieving his health goals.

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Ongoing Care Plans within OASIS date (04/08/2026):

Problems	Onset Date	Status	Initiated From
Vital Signs Assessment	04/08/2026	**0** out of **1** Met Goal(s)	04/08/2026 - 06/06/2026

Target Completion: *1* **Term:** *Long-term* **Status:** *Unmet* **Unmet on:** No Data

Goal:
Patient's vital signs will be within normal limits.

Intervention:
Check VS every visit and report to PMD if patient vitals are not within the parameters.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Problems	Onset Date	Status	Initiated From
Hypertension	04/08/2026	**0** out of **1** Met Goal(s)	04/08/2026 - 06/06/2026

Target Completion: *1* **Term:** *Short-term* **Status:** *Unmet* **Unmet on:** No Data

Goal:
Maintain blood pressure within normal range (120/80 mmHg).

Intervention:
Teach the patient and family about dietary modifications, including low sodium diet.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Assist with medication management, ensuring adherence to prescribed antihypertensive medications.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Teach relaxation techniques to reduce stress and its impact on blood pressure.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Assist with regular blood pressure monitoring at home.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Teach the importance of regular physical activity and assist in developing an exercise plan.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Diabetes Mellitus (Type 1 and Type 2) 04/08/2026 **0** out of **1** Met Goal(s) 04/08/2026 - 06/06/2026

Target Completion: *1* **Term:** *Short-term* **Status:** *Unmet* **Unmet on:** No Data

Goal:
Maintain blood glucose levels within target range (70-130 mg/dL before meals).

Intervention:
Teach the patient and family about carbohydrate counting and meal planning.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Assist with blood glucose monitoring and insulin administration.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Teach the patient about recognizing and managing hypoglycemia and hyperglycemia.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Spondylosis without myelopathy or radiculopathy, lumbar region 04/08/2026 **0** out of **1** Met Goal(s) 04/08/2026 - 06/06/2026

Target Completion: *1* **Term:** *Short-term* **Status:** *Unmet* **Unmet on:** No Data

Goal:
Reduce pain and improve mobility in the lumbar region.

Intervention:
Assist with physical therapy exercises focused on strengthening and flexibility.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Teach proper body mechanics and ergonomics for daily activities.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Assist with the application of heat or cold therapy to alleviate pain.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Bilateral primary osteoarthritis of knee 04/08/2026 **0** out of **1** Met Goal(s) 04/08/2026 - 06/06/2026

Target Completion: *1* **Term:** *Short-term* **Status:** *Unmet* **Unmet on:** No Data

Goal:
Enhance joint function and reduce pain in the knees.

Intervention:
Create a tailored exercise regimen focusing on low-impact activities.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Teach the patient and family how to use assistive devices correctly.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Assist with weight management strategies to alleviate stress on the knees.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Provide education on pain management techniques, including medication adherence.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Assist with home modifications to enhance mobility and safety.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Adhesive capsulitis of left shoulder 04/08/2026 **0** out of **1** Met Goal(s) 04/08/2026 - 06/06/2026

Target Completion: *1* **Term:** *Short-term* **Status:** *Unmet* **Unmet on:** No Data

Goal:
Increase range of motion and decrease pain in the left shoulder.

Intervention:
Implement a progressive stretching program for the shoulder.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:

Discontinue Date / Date Resolved: —

Teach the patient and family about the stages of healing and the importance of adherence to the exercise program.

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Intervention:
Assist in applying modalities, such as ice or heat, as appropriate.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Rehospitalization Risk and Medication Management | 04/08/2026 | **0** out of **1** Met Goal(s) | 04/08/2026 - 06/06/2026

Target Completion: *1* **Term:** *Short-term* **Status:** *Unmet* **Unmet on:** No Data

Goal:
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.

Intervention:
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.

Discontinue Date / Date Resolved: —

Assigned to Staff Type: ☑ SN

Education, Teaching or treatment for the identified problems	Education Status	Date	Accomplished On
No Progress Yet.			

Assigned Staff: Rudich, Charlotte RN

Electronically Signed by Rudich, Charlotte RN 04/08/2026